



CARTI Cancer Center

8901 Carti Way, Little Rock, AR 72205

Phone: 501-906-3000

Patient Name: **Monson, Berdia M**

Date: **4/28/2026**

Patient Number: **1095086**

Date Of Birth: **5/1/1937**

INITIAL ONCOLOGY OFFICE VISIT

FOLLOW UP VISIT

Chemotherapy

Chief Complaint

Evaluation and management of: Uterine Carcinosarcoma

Problems

Date	Type	ICD-9	ICD-10	Description	Disease Status	Status Date
3/10/2026	Primary	182.0	C54.1	Uterine (Endometrial), Carcinoma (Gynecologic, Uterine (Endometrial) Cancer) - Clinical Stage IVB (AJCC v8) TNM: cM1		

History of Present Illness

She is here today for her follow up visit.

Past Medical History

Date	Type	ICD-9	ICD-10	Problem	Comment	Status
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Surgical History

Triple Bypass 2020

Cardiac Stent Placement x 2 prior to bypass

Allergies

codeine, Penicillins

Medications

Inside	Drug	Script Date	Qty	Rfls	Instructions
	albuterol sulfate HFA 90 mcg/actuation aerosol inhaler	3/10/2026		0	2 puffs q. 4 hours prn
	amlodipine 10 mg tablet	3/10/2026		0	Take 1 by mouth daily.

	aspirin 81 mg chewable tablet	3/10/2026		0	Take 1 by mouth daily.
	black seed 4.5 gram/5 mL oral oil	3/10/2026		0	Take by mouth daily.
	calcium 600 mg (as carbonate)- vitamin D3 10 mcg (400 unit) tablet	3/10/2026		0	Take 1 by mouth daily.
	cyanocobalamin (vit B-12) 500 mcg tablet	3/10/2026		0	Take 1 by mouth daily.
	ferrous sulfate 325 mg (65 mg iron) tablet	3/10/2026		0	Take 1 by mouth daily.
	FLUTICASONE 50MCG NASAL SPR	4/6/2026	48	0	USE 1 SPRAY(S) IN EACH NOSTRIL TWICE DAILY
Y	lidocaine-prilocaine 2.5 %-2.5 % topical cream	4/21/2026	30	0	apply to skin over port area 30 minutes prior to access, cover with a clear plastic wrap
	losartan 100 mg- hydrochlorothiazide 25 mg tablet	3/10/2026		0	Take 1 by mouth daily.
	megestrol 40 mg tablet	3/10/2026		0	Take 1 by mouth daily.
	metoprolol tartrate 100 mg tablet	3/10/2026		0	Take 1 by mouth daily.
	MINOXIDIL 10 MG TABS	2/25/2026	30	0	
	MINOXIDIL 10MG TAB	2/25/2026	30	0	TAKE 1 TABLET BY MOUTH ONCE DAILY
	omeprazole 40 mg capsule, delayed release	3/10/2026		0	Take 1 by mouth daily.
	rosuvastatin 5 mg tablet	3/10/2026		0	Take 1 by mouth daily (CURRENTLY HOLDING)

Immunization Verification

- Patient reports **NOT** being vaccinated for COVID.

Family History

Daughter - Breast Cancer

Social History

Patient denies any prior alcohol use.

Denies any recreational drug use.

Patient is widowed.

Patient lives alone, recently living with her daughter.

Patient has children.

Tobacco Use and Cessation Counseling

Non smoker.

On-Going Treatment Events

Date	Type	Details	Outcome	Comment
1/1/2024	Other Therapies	Endoscopic examination of colon		

3/10/2026	Other Therapies	Mammogram		Unsure of date
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(CQM) Patient Stated Pain:

Patient Stated Pain Level 0 - No pain.

1. Little interest or pleasure in doing things

0 - Not at all

2. Feeling down, depressed, or hopeless

0 - Not at all

PHQ2 Scoring (Questions 1-2)

PHQ2 Score = 0

Vitals Signs

Vitals on 4/28/2026 8:21:00 AM: Height=154.9cm, Weight=81.7kg, Temp=98.1f, Heart rate=64bpm, Respiration rate=17, SystolicBP=156, DiastolicBP=**62**, Pulse Ox=93%, Comment=EH

Performance Status

ECOG 2: Ambulatory and capable of all self care but unable to carry out any work activities. Up and about more than 50% of waking hours

Physical Examination

Physical Examination

General: Well developed, well nourished patient, in no acute distress. Patient appears stated age.

Skin: No abnormal skin lesions noted.

Head: Normocephalic.

Eyes: Sclerae are anicteric.

Ears, Nose, Throat, and Mouth: Normal oral mucosa and oropharynx.

Neck: Neck is supple. No cervical masses present. Trachea is midline. No thyromegaly.

Right Breast: Breast exam deferred.

Left Breast: Breast exam deferred.

Lungs: Clear to auscultation. No abnormal breath sounds or rubs.

Cardiac: normal rate; regular rhythm. S1/S2 without murmurs.

Abdomen: Abdomen is soft, nontender, nondistended, without masses. No hepatomegaly. No splenomegaly. No ascites present.

Bowel sounds active.

Hematologic/Lymphatic: No petechiae. No purpura. No cervical, axillary, or inguinal LAN.

Extremities: No edema. No cyanosis. No digital clubbing. No discoloration.

Musculoskeletal: Normal range of motion. Strength and tone are normal. Back and Spine non-tender to palpation and percussion.

Genitourinary: Deferred.

Rectal/Pelvic Exam: Deferred.

Neurologic: No focal findings

Laboratory Results

Lab results on 4/28/2026: WBC=6.28 10³/uL, RBC=**3.52** 10⁶/uL, Hgb=**9.8** g/dL, HCT=**30.6** %, MCV=86.9 fL, MCH=27.8 pg, MCHC=32.0 g/dL, Platelet #=**70** 10³/uL, RDW Ratio=14.7 %, MPV=11.4 fL, Neu # (ANC)=4.83 10³/uL, Neut %=76.9 %, Lymph#=**0.52** 10³/uL, Lymph%=**8.3** %, MONO#=0.39 10³/uL, MONO%=6.2 %, EOS#=0.49 10³/uL, EOS%=**7.8** %, BASO#=0.03 10³/uL, BASO%=0.5 %, IG#=0.02 10³/uL, IG%=0.3 %, Sodium=137 mmol/L, Potassium=3.5 mmol/L, Chloride=103 mmol/L, CO2=25 mmol/L, Glucose=**112** mg/dL, BUN=14 mg/dL, Creat=0.9 mg/dL, Calcium=8.8 mg/dL, Total Protein=7.4 g/dL, Albumin=3.7 g/dL, A/G=1.0 (CALC), Total Bili=0.6 mg/dL, Alk Phos=81 U/L, ALT=14.0 U/L, AST=22 U/L, eGFR=63.2 , T4 Free=1.2 ng/dL, TSH=1.78 uIU/mL Orders scheduled for today: CBC,PLT,DIFF, CMP, T4, Free, TSH

- Laboratory results from previous and interim visit were reviewed and discussed with the patient.

Imaging Results

Print? ☐	Date of Doc.	Name	MD Interpretation	Comment
☐	4/8/2026	Scheduling Call-No answer		
☐	4/8/2026	Sedation Clearance Request		
☐	4/8/2026	Scheduling Call- Needs clearance		
☐	4/20/2026	IVR Procedure Confirmation Call		
☐	4/21/2026	IVR Docs		
☐	4/21/2026	ECG		
☐	4/21/2026	IVR PORT PLACEMENT/INSERTION TUNNEL CV CATH W SUBQ PORT		

There are no radiology reports that are pertinent for this patient's visit.

Assessment

Mrs. Berdia Monson is a 88 year old with past medical history significant for hypertension, hyperlipidemia, and CAD. She is referred here by Lisa Bazzett Matabele, MD. She is here for further workup and evaluation of Uterine Carcinosarcoma.

She is s/p hysterectomy with BSO on 1/14/26.

Endomyometrium:

- Carcinosarcoma, invasive approximately 70% into myometrium.
- Positive for extensive lymphovascular invasion.
- See synoptic report.

Cervix:

- Superficial involvement by carcinosarcoma extending to the anterior inked cauterized edge.
- No stromal involvement identified

Serosa: No significant histologic change

Bilateral Ovaries: Carcinosarcoma present within the lymphovascular spaces of bilateral ovaries

Bilateral Fallopian Tubes: Benign fallopian tubes with no significant histopathologic change

Pelvic Washing: Negative for malignant cells

PET/CT Skull Base to Mid Thigh 2/17/26

1. Patient status post hysterectomy bilateral salpingo oophorectomy. Thickening and intense uptake at the left vaginal cuff is concerning for malignancy.
2. Hypermetabolic bilateral pelvic nodules/lymph nodes consistent with metastatic disease. Multiple hypermetabolic retroperitoneal lymph nodes consistent with metastatic.
3. Intense uptake associated with thickening small bowel segment right abdomen is indeterminate and may reflect bowel inflammation can not exclude underlying malignancy.

3/10/26: She is here today for her initial visit. She started having postmenopausal bleeding, reports this started about 6-8 months ago. She is s/p hysterectomy with BSO with pathology showing uterine carcinosarcoma. She is here today with her daughter today.

- Reviewed and discussed pathology results showing uterine carcinosarcoma
- Reviewed and discussed PET scan findings showing uptake at the left vaginal cuff, bilateral pelvic nodules/lymph nodes, retroperitoneal lymph nodes consistent with metastatic disease. Also noting intense uptake associated with thickening small bowel segment of the right abdomen.
- I spoke to the patient at length about possible diagnosis, differential diagnosis, workup, treatment, and answered all the questions.
- I discussed treatment options of Carbo + Taxol + Dostarlimab, as this is a very intense chemotherapy.
- Will decrease the dose of carboplatin and Taxol. Will start with immunotherapy and add this chemotherapy at low doses later
- I will request insurance approval for immunotherapy +/- chemotherapy, plan to start in 1-2 weeks

3/12/26 CT CAP:

1. There are findings concerning for evidence of recurrent/growing implant disease of lower left pelvic sidewall/vaginal cuff margin.
2. There are several enlarging lymph nodes of abdomen and pelvis level are concerning for growing metastasis.
3. No evidence metastatic disease at chest level.

3/17/26: Denies pain. Denies fevers or signs of infection. Denies issues with bleeding. She has been having some constipation. Has questions about a port.

- Begin Dostarlimab today and q 3weeks
- She would not tolerate CarboTaxol. I will try to reduce her lower dose at lateral. She did not interested in getting any side effects chemotherapy at this time. Will proceed with immunotherapy only. I might add Lenvima at low-dose
- CT CAP W results discussed today, showing several enlarging lymph nodes of abdomen and pelvis level are concerning for growing metastasis
- Refer to Dr. Cameron for radiation planning
- Discussed starting chemotherapy after visit with Dr. Cameron and first treatment
- Discussed waiting on port, to see if chemotherapy will be added

3/31/26 MRI Pelvis:

Indeterminate asymmetric soft tissue changes involving the left vaginal cuff, with 2 additional mildly prominent nodes identified within the pelvis as described. In the setting of an indeterminate MRI examination, PET/CT would offer more definitive characterization as to whether these areas are metabolically active and therefore concerning for recurrence and possible metastatic adenopathy.

4/7/26: She is doing well, no new concerns at this time. She is here with her daughter today

- Continue Dostarlimab today and q 3weeks
- Begin XRT with Dr. Cameron 4/9/26
- Discussed adding Lenvima after radiation is complete

4/28/26: She started radiation of 28fx on 4/9/26. She is feeling okay, no new complaints. She is tolerating treatment well. She is here with her daughter today.

- **Uterine Carcinosarcoma** s/p hysterectomy with BSO on 1/14/26.
- **Persistent disease in the lymph nodes, metastatic**
- **Status post hysterectomy and bilateral salpingo-oophorectomy**
- **On palliative radiation**
- **On immunotherapy**

- Check CBC w/diff, CMP, LDH. Labs reviewed
- Continue Dostarlimab only today and q 3weeks
- Continue XRT with Dr. Cameron, began 4/9/26
- Discussed adding Lenvima after radiation is complete
- RTC in 3 weeks for labs, exam, and treatment

Provider Attestation

I personally performed the services described in this documentation. All medical record entries made by the clinical staff were at my direction and in my presence. I have reviewed the chart and agree that the record reflects my personal performance and is accurate and complete.

Goal of Treatment

- Palliative

Education and Teaching

- Discussed diagnosis and goals of treatment.
- Discussed plans for follow-up visits, lab and other testing.

Visit Location

CARTI Pine Bluff Cancer Center
5001 Bobo Road
Pine Bluff, AR 71603

Signed



Fax: 501-907-8360

Appalanaidu Sasapu, MD on 4/29/2026 at 1:18 PM